

Food Insecurity

TC: C-1164 (TC)

[Link to Codes](#)

MCG Health
Transitions of Care
28th Edition

- Assessment
- Plan of Care
- Evidence Summary
- References
- Codes

Assessment

Q1. Patient: In the past year, have you or any family members you live with been unable to get food?

- Yes [Q3] [B1] [P1]
- No
- Unknown

Q2. Patient: In the past year, have you been unable to eat healthy because of the cost of food?

- Yes [Q3] [B1] [P1]
- No
- Sometimes [Q3] [B1] [P1]
- Unknown

Q3. Patient: Do you need help with any of the following? (Check all that apply.)

- Finding food resources [P1]
- Filling out paperwork/electronic forms [P1]
- Following up on existing referrals [P1]
- Meeting medical needs due to food insecurity [P1]
- Declined
- Not applicable

Plan of Care

Barriers

B1. Food insufficient

Problems

P1. Food insecurity present

- **Goal:** New food resources obtained or existing resources modified so they are adequate, stable, and safe(1) **NN**
 - **Interventions:**
 - **ASSIST:** Identifying food resource options
 - **ASSIST:** Completing documentation for food assistance, if needed
 - **ASSIST:** Appointment scheduling with provider if financial problems negatively affecting self-care
 - **ASSIST:** Identifying available food resources and contacting for support(2)
 - **EDUCATE:** Availability of resources and interventions varies by region.
 - **EDUCATE:** Types of food resources available; examples include(3)(4)(5):
 - Community programs (eg, Meals on Wheels, faith-based organizations)
 - Federal assistance programs (eg, Women, Infants, and Children (WIC), Supplemental Nutrition Assistance Program (SNAP))
 - Food vouchers
 - Medically tailored meal programs(6)
 - **EDUCATE: Resources:**
 - Benefit Finder: www.benefits.gov
 - Findhelp: www.findhelp.org
 - Food and Nutrition Service (FNS) Nutrition Programs: www.fns.usda.gov/programs
 - United Way: www.211.org
 - **COORDINATE:** Social worker referral, as needed(7)(8)

- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Follow-up contact to evaluate food status, referral status, and need for further assistance(9)(10)
- **Goal:** Medical needs being met while food insecurity present
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Identifying factors that might impact food needs and storage (eg, housing insecurity, transportation needs)
 - **EDUCATE:** Risks of improper storage or preparation of foods (eg, too much water in baby formula, expired foods)
 - **EDUCATE:** Food insecurity's impact on chronic condition (eg, diabetes, anemia)(11)
 - **EDUCATE:** Role of community and medical services to support self-care
 - **COORDINATE:** Increased intervals of follow-up with preferred method of communication (eg, phone, text, email)
 - **COORDINATE:** Social worker referral, as needed(7)(8)
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** Follow-up contact to evaluate food status, referral status, and need for further assistance(9)(10)
 - **SEND:** Eating healthy on a budget  (Spanish)

Evidence Summary

Quality standards: Standards indicate that program content should address psychosocial issues (eg, inadequate food) that would impede self-management. Programs should have a process for following up with patients on the status of referrals.(12)

Evidence: In a cross-sectional study of 288,079 families, people older than 65 years were 39% less likely to report food insecurity compared to those age 18 to 34 years. Women were 23% more likely to be food insecure than men; non-Hispanic Black persons were 1.7 times more likely to be food insecure than non-Hispanic White persons; and a clear gradient existed by income, with lower-income people more likely to be food insecure.(13)

References

1. Davidson KW, et al. Developing primary care-based recommendations for social determinants of health: methods of the U.S. Preventive Services Task Force. *Annals of Internal Medicine* 2020;173(6):461-467. DOI: 10.7326/M20-0730. [Context Link 1] View abstract...
2. Steeves-Reece AL, Totten AM, Broadwell KD, Richardson DM, Nicolaidis C, Davis MM. Social needs resource connections: a systematic review of barriers, facilitators, and evaluation. *American Journal of Preventive Medicine* 2022;62(5):e303-e315. DOI: 10.1016/j.amepre.2021.12.002. [Context Link 1] View abstract...
3. Greenwald HP, Tao E, Tilley G. Farmers' market incentives for low-income families: who uses, how much, and why. *American Journal of Preventive Medicine* 2022;62(6):864-871. DOI: 10.1016/j.amepre.2021.11.023. [Context Link 1] View abstract...
4. Merchant T, et al. Healthcare-based interventions to address food insecurity during pregnancy: a systematic review. *American Journal of Obstetrics and Gynecology MFM* 2023;5(5):Online. DOI: 10.1016/j.ajogmf.2023.100884. [Context Link 1] View abstract...
5. Oronce CIA, Miake-Lye IM, Begashaw MM, Booth M, Shrank WH, Shekelle PG. Interventions to address food insecurity among adults in Canada and the US: a systematic review and meta-analysis. *JAMA Health Forum* 2021;2(8):Online. DOI: 10.1001/jamahealthforum.2021.2001. [Context Link 1] View abstract...
6. Berkowitz SA, Terranova J, Randall L, Cranston K, Waters DB, Hsu J. Association between receipt of a medically tailored meal program and health care use. *JAMA Internal Medicine* 2019;179(6):786-793. DOI: 10.1001/jamainternmed.2019.0198. [Context Link 1] View abstract...
7. McQueen A, et al. Social needs, chronic conditions, and health care utilization among Medicaid beneficiaries. *Population Health Management* 2021;24(6):681-690. DOI: 10.1089/pop.2021.0065. [Context Link 1, 2] View abstract...
8. Gonzalez JV, Hartford EA, Moore J, Brown JC. Food insecurity in a pediatric emergency department and the feasibility of universal screening. *Western Journal of Emergency Medicine* 2021;22(6):1295-1300. DOI: 10.5811/westjem.2021.7.52519. [Context Link 1, 2] View abstract...
9. Care coordination and care transitions to support adherence. In: Perez R, Rogers S, Prince M, Fraser K, editors. *Case Management Adherence Guidelines*. 2020 ed. Case Management Society of America; 2022:43-56. [Context Link 1, 2]
10. Monitoring and evaluation. In: Gillingham DC, editor. *Foundations of Case Management*. Blue Bayou Press; 2021:51-52. [Context Link 1, 2]
11. Hill-Briggs F, et al. Social determinants of health and diabetes: a scientific review. *Diabetes Care* 2020;44(1):258-279. DOI: 10.2337/dci20-0053. [Context Link 1] View abstract...
12. Standards of Practice for Case Management. [Internet] Case Management Society of America. 2022 Accessed at: <https://www.cmsa.org/>. [accessed 2023 Sep 21] [Context Link 1]
13. Walker RJ, Garacci E, Dawson AZ, Williams JS, Ozieh M, Egede LE. Trends in food insecurity in the United States from 2011-2017: disparities by age, sex, race/ethnicity, and income. *Population Health Management* 2021;24(4):496-501. DOI: 10.1089/pop.2020.0123. [Context Link 1] View abstract...

Codes

ICD-10 Diagnosis: Z58.6, Z59.41, Z59.48

MCG Health
Transitions of Care 28th Edition
Copyright © 2024 MCG Health, LLC
All Rights Reserved

Last Update: 3/14/2024 4:18:48 AM
Build Number: 28.1.20240313235330.016020